SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Weekly carboplatin + paclitaxel with radiotherapy

Stage III non-small-cell lung cancer · concurrent chemoradiation · NCCP 00309 · version 6a · 7-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
Chemotherapy is given alongside radiotherapy to treat stage III non-small-cell lung cancer when cisplatin is unsuitable. Your team explains the treatment aim and choices.

Schedule
Both medicines are given by drip once every 7 days for seven weeks, alongside a separate radiotherapy course. The plan may change for side effects or test results. Further chemotherapy after radiotherapy is a separate decision.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Paclitaxel
TAXANE CHEMOTHERAPY

- ☐ Numbness / tingling that may persist
- ☐ Infusion allergy
- ☐ Low blood counts / infection
- ☐ Hair loss
- ☐ Muscle / joint pain

Carboplatin
PLATINUM CHEMOTHERAPY

- ☐ Low blood counts / serious infection
- ☐ Bleeding / bruising
- ☐ Nausea and vomiting
- ☐ Infusion allergy

Discuss serious infection/sepsis and possible hospital admission, bleeding, infusion reactions, lung inflammation, painful swallowing/dehydration, fatigue, fertility, hair loss and individual material risks. Paclitaxel neuropathy may persist; discuss muscle/joint pain, renal dose checks and late respiratory symptoms after chest radiation. Radiotherapy needs its own informed-consent discussion; chemotherapy consent does not replace it.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00309 v6a.

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